

Histology: Histology of Connective Tissue, Muscle, Nerve, and Applications

Brian Beatty, PhD
Anatomy
bbeatty@nyit.edu

Session objectives

1. Describe and compare the general morphology, functions, and locations of cartilage, bone, and blood.
2. Compare and contrast the characteristics, function, and location of various types of muscle tissue.
3. Compare and contrast the function and morphology of various types of nervous tissue.
4. Explain histogenesis and development of different types of tissue.
5. Apply principle by identifying various tissue types.

Specialized Connective Tissue

- **Specialized C.T.:**

1. **Cartilage**

1. Hyaline
2. Elastic
3. Fibro

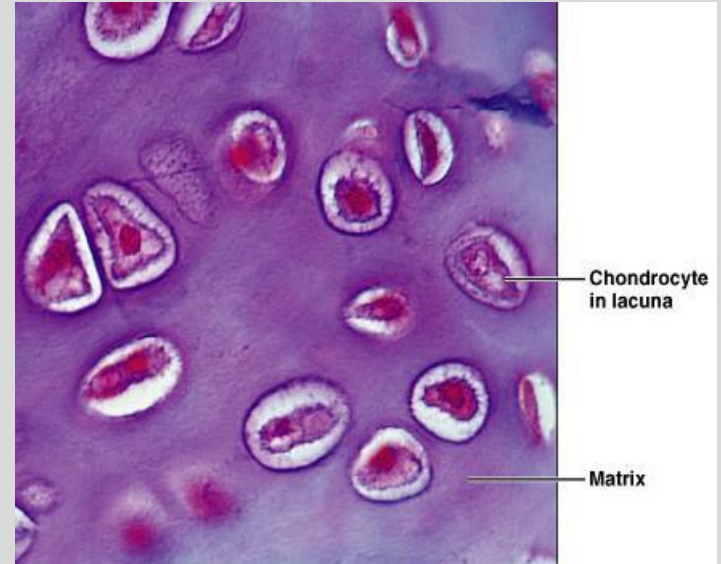
2. **Bone**

1. Spongy
2. Compact

3. **Blood**

Cartilage

- Characteristics:
 - a) Firm & durable (mechanical stress)
 - b) Matrix 60-80% water
 - c) Type II Collagen & elastic fibers
 - d) High conc. of GAGs & proteoglycans
 - e) Avascular (not innervated)
 - f) Development/growth of long bones
 - g) Chondrocytes in lacunae
 - h) Covered perichondrium
- Types:
 1. Hyaline – most common
 2. Elastic – ears, larynx
 3. Fibrocartilage – vertebral discs, symphysis pubis, cranial sutures

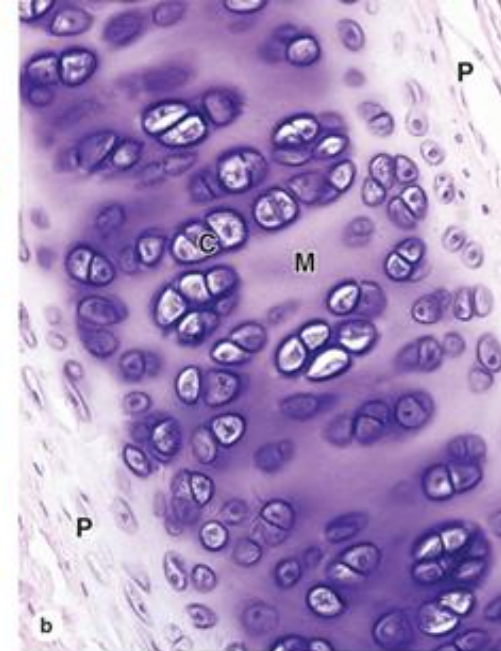
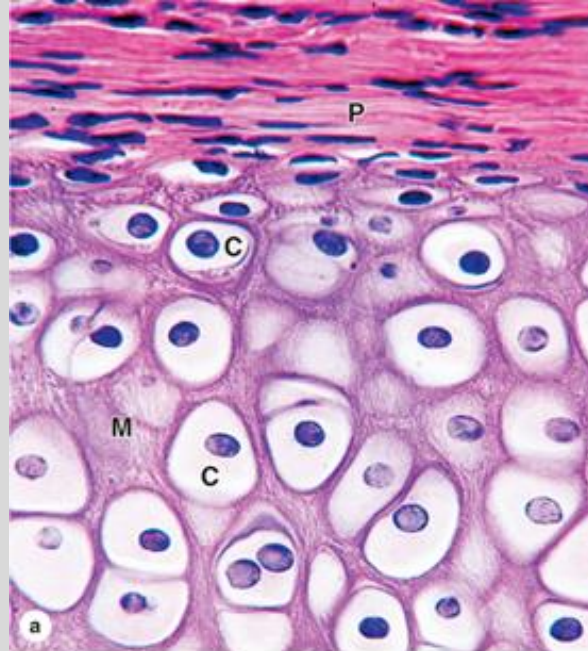


Hyaline Cartilage

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- Description
 - a) Hyaline = glassy
 - b) homogeneous & semitransparent
- Function
 - a) Supports & reinforces
 - b) Resilient cushion
 - c) Resists repetitive stress
- Location
 - a) Articular surfaces of movable joints
 - b) Walls of larger respiratory passages
 - c) Epiphyseal plates
- Loss → osteoarthritis
- Costal cartilage calcify with age
- Somatotropin (anterior pituitary)



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Histology: Text and Atlas, 15th Edition.
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Elastic cartilage

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- Description
 - a) Similar to hyaline cartilage + more elastic fibers
- Function
 - a) Maintains shape of structure
 - b) Allows great flexibility
- Location
 - a) Supports external ear
 - b) Epiglottis
 - c) Upper respiratory tract

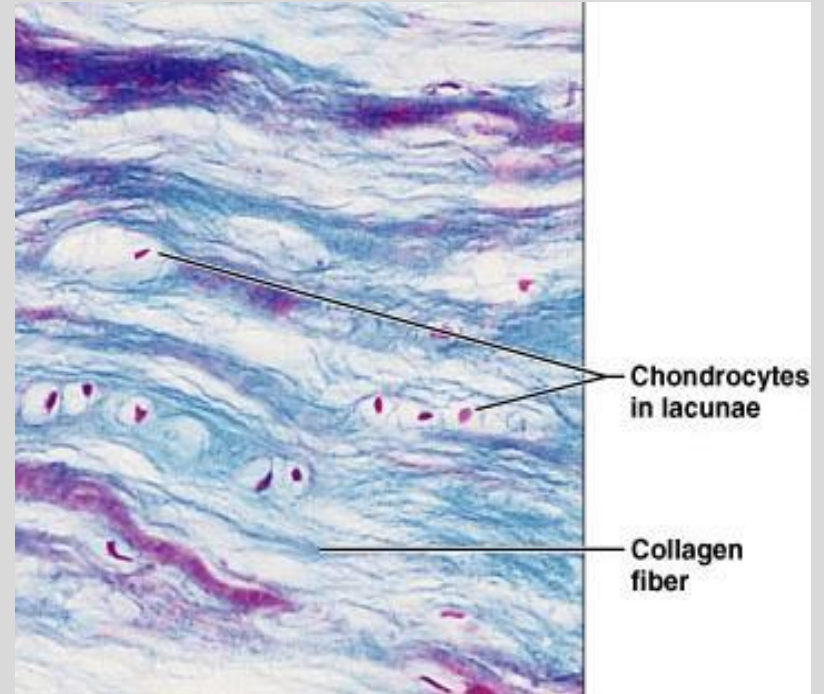


Fibrocartilage

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- Description
 - i. Hyaline cartilage & dense connective tissue
- Function
 - i. Tensile strength & ability to absorb compressive shock
- Location
 - i. Intervertebral discs
 - ii. Pubic symphysis

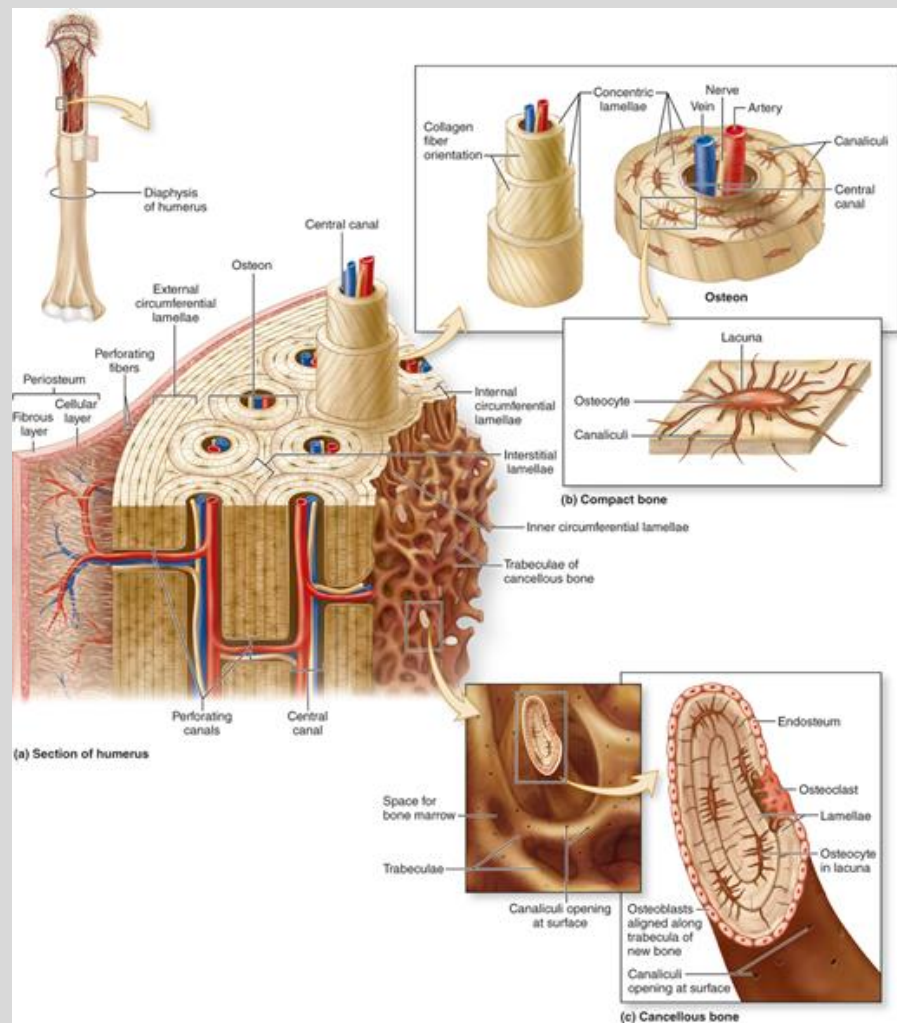


Bone

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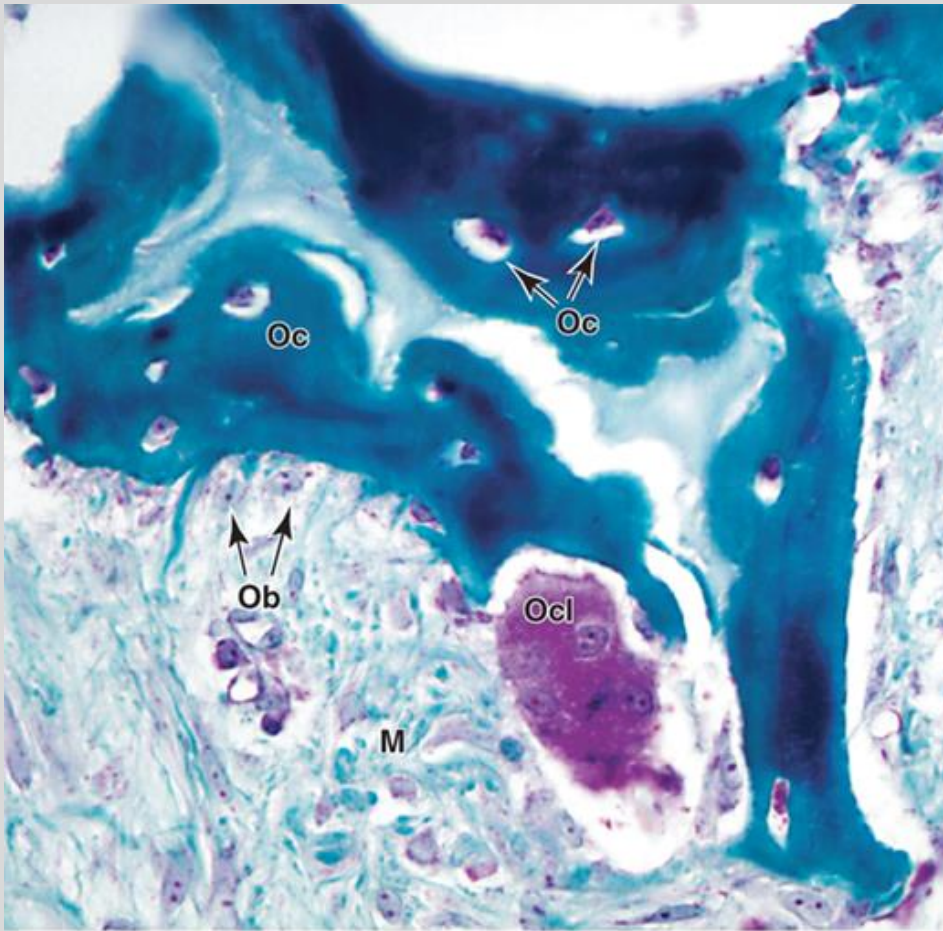
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- Calcified matrix
- Function
 - a. Supports & protects organs
 - b. Provides levers & attachment site for muscles
 - c. Stores Ca^{++} & other minerals
 - d. Stores fat
 - e. Red marrow (site for blood cell formation)



Bone cells

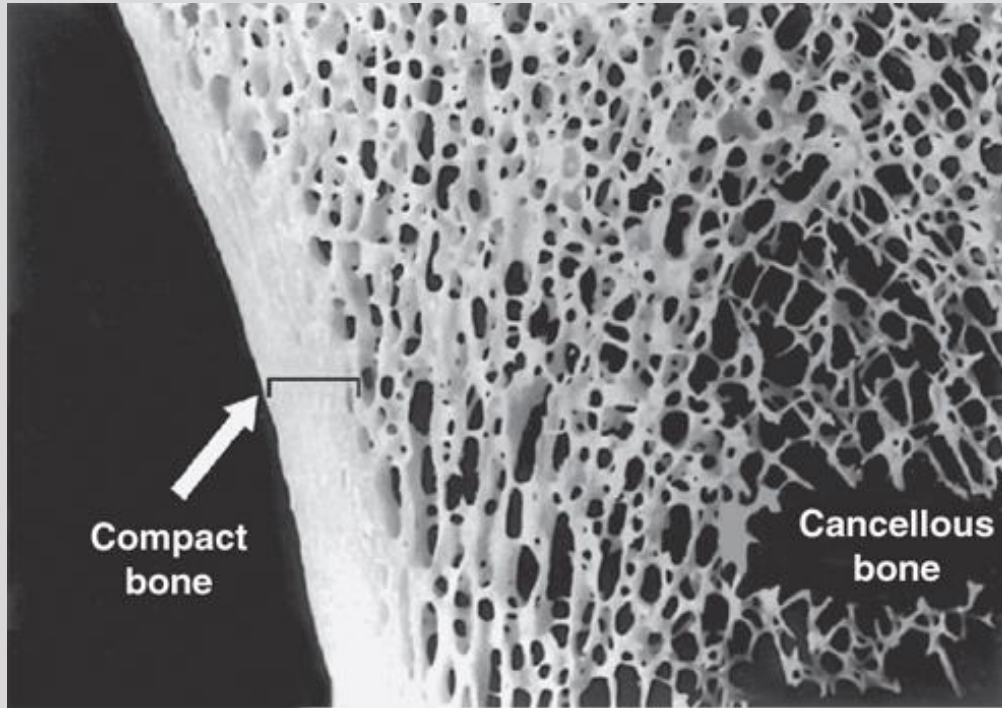
- 3 types of cells:
 1. Osteocytes
 2. Osteoblasts
 3. Osteoclasts



Bone types

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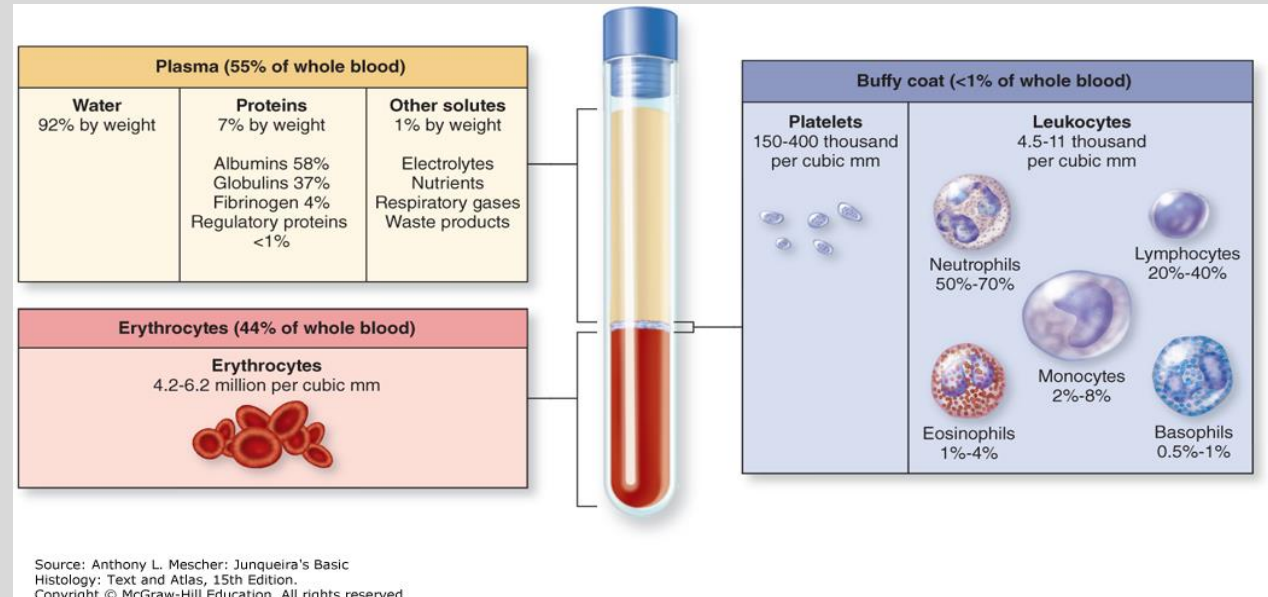


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- 2 types:
 1. Compact (dense)
 2. Trabecular (spongy; light)

- Transport: O₂, CO₂, metabolites, hormones
- Heat distribution
- Regulation of body temperature
- Maintenance of acid-base & osmotic balance

Blood



Muscle Tissue

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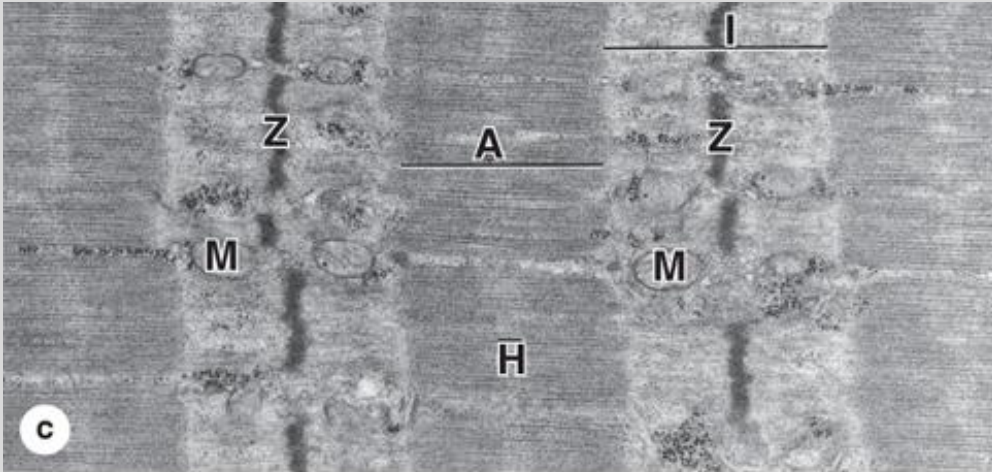
- Function: Movement & heat production
- Types
 1. Skeletal
 2. Cardiac
 3. Smooth

Skeletal Muscle

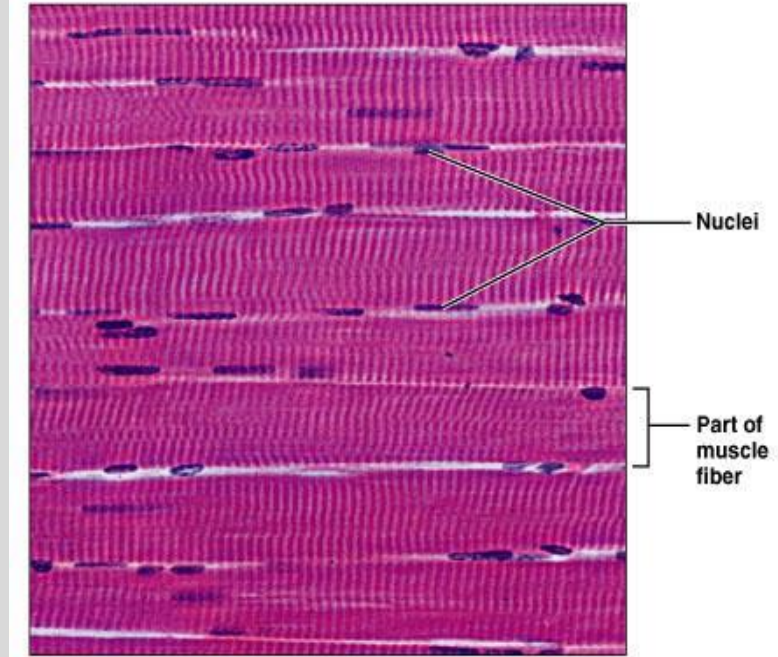
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- Attached to bones
- Long, multinucleated cylindrical cells
- Mostly voluntary
- Striated
- Sarcomere

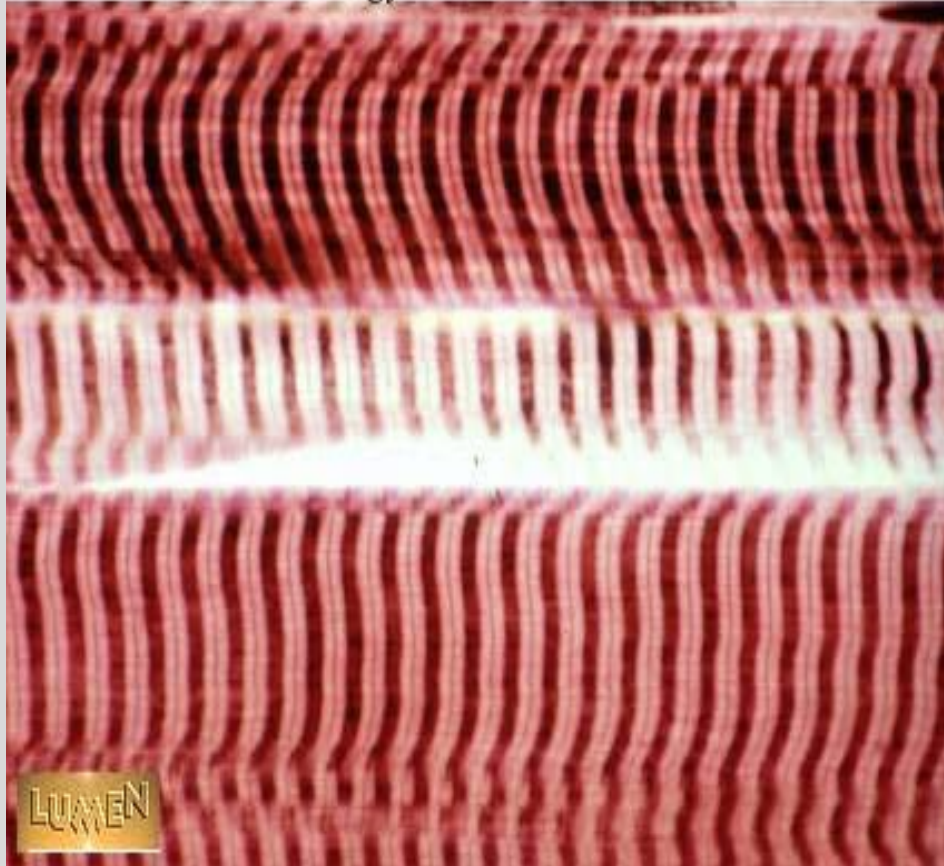


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Skeletal muscle

Histology Lab Part 7: Slide 46



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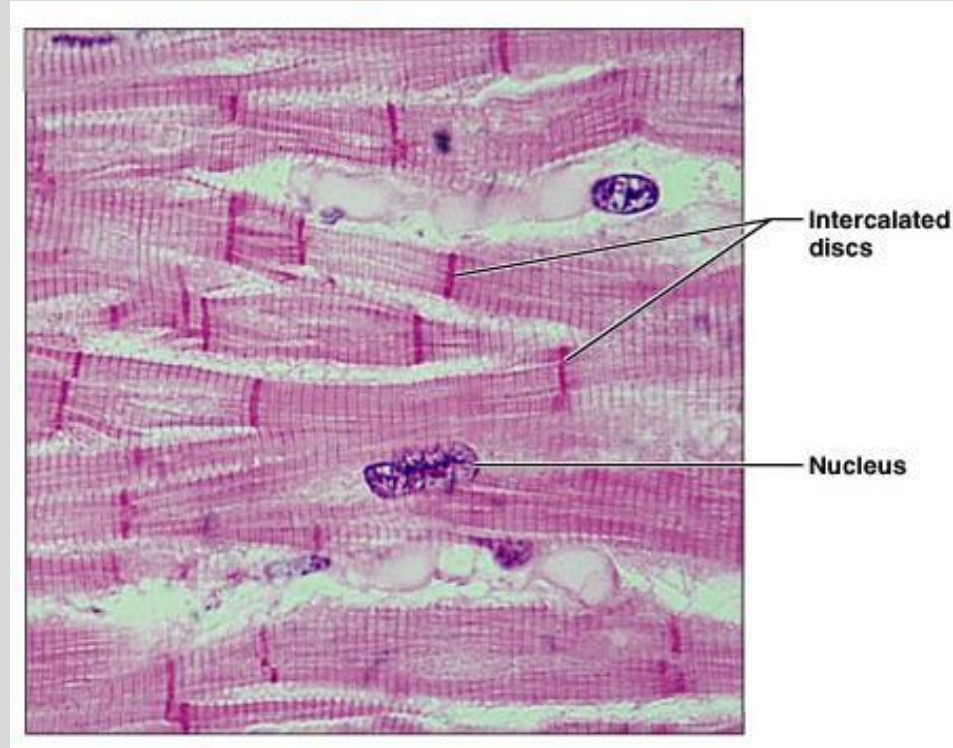
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Cardiac Muscle

- Uni or bi-nucleated, branching cells (myocardium)
- Striated
- Intercalated discs
- Involuntary
- Autonomous

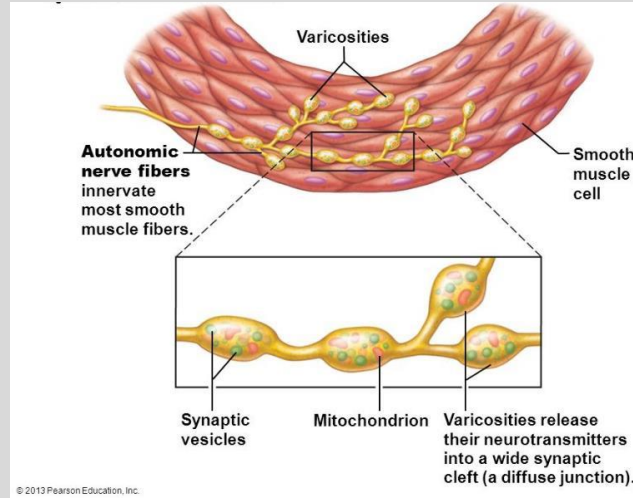
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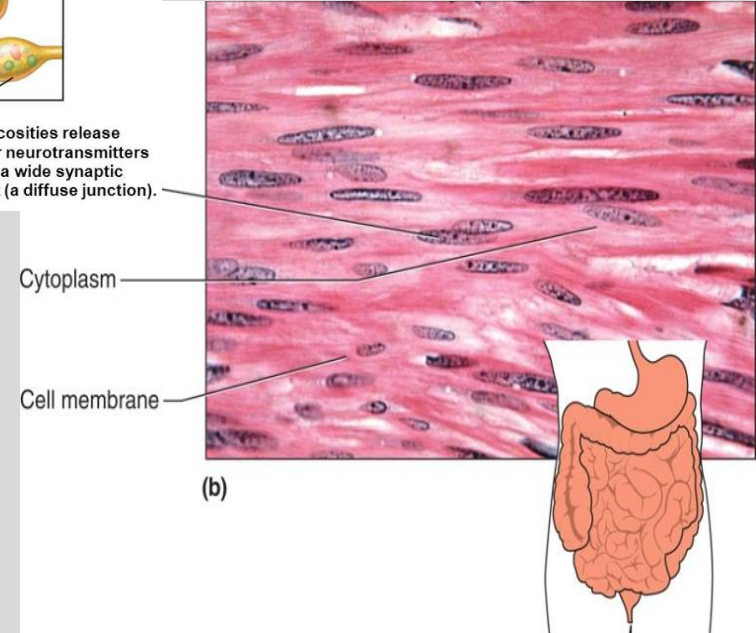
Smooth Muscle

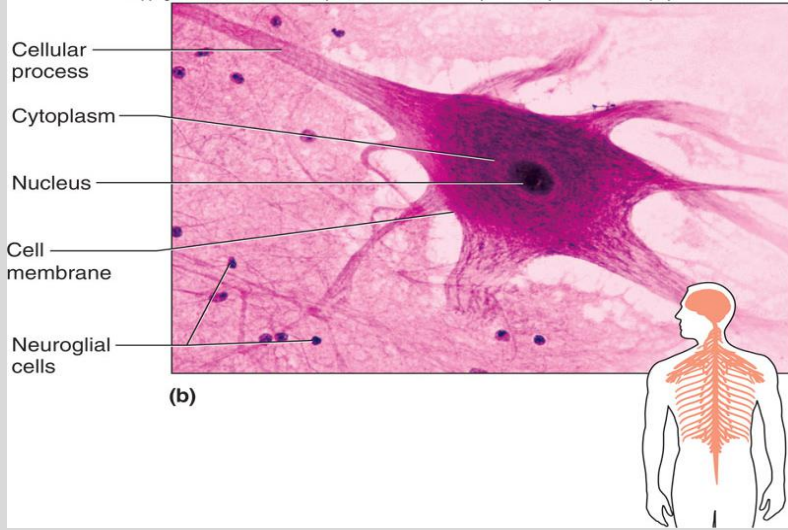
- Spindle-shaped cells with central nuclei
- No striations (no sarcomeres)
- Involuntary
- Autonomous
- Walls of hollow organs & blood vessels
- Neuromuscular junction is distinctive (varicosities)



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Nervous Tissue

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• Neurons & neuroglial cells

a) Neurons:

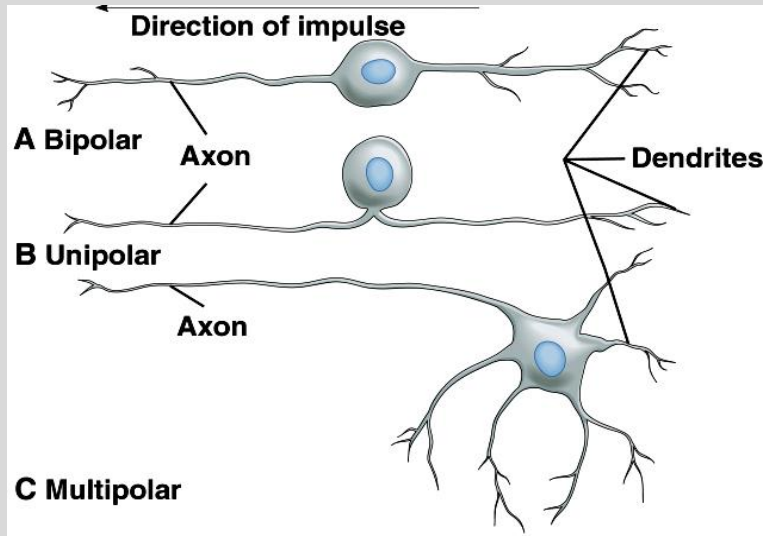
- i. Processes: axons & dendrites
- ii. Uni, bi & multipolar
- iii. Sensory, motor & interneurons

b) Glial cells:

- i. CNS: Astrocytes, oligodendrocytes, microglia & ependymal cells
- ii. PNS: Schwann & satellite cells

• CNS: brain & spinal cord

• PNS: cranial & spinal nerves



Tissues throughout life

- Early on – Gastrulation
 - a) Most important time in life!!
 - a) When tissues differentiate – problems here affect development
- At end of second month of development
 - a) Primary tissue types have appeared
 - b) Major organs are in place
- Adulthood
 - a) Only a few tissues regenerate
 - b) Many tissues still retain populations of stem cells
- With increasing age:
 - a) Epithelia thin
 - b) Collagen decreases
 - c) Bones, muscles, & nervous tissue begin to atrophy
 - d) Poor nutrition & poor circulation – poor health of tissues
 - e) Increased chance of developing cancer

Histogenesis

- Trilaminar embryo – ectoderm, mesoderm, & endoderm
- Ectoderm – outermost layer
 - 1) Surface ectoderm: epidermis & derivatives, cornea, lens epithelia, enamel, internal ear, adenohypophysis, mucosa of oral & anal cavities
 - 2) Neuroectoderm: CNS & PNS
- Mesoderm – middle layer
 - 1) All connective tissue
 - 2) All muscle tissue
 - 3) Heart, blood vessels, & lymphatics
 - 4) Spleen
 - 5) Kidneys
 - 6) Gonads
 - 7) Mesothelium
 - 8) Adrenal cortex

Histogenesis

- Endoderm – innermost layer
 - 1) Alimentary canal epithelium
 - 2) Liver, pancreas, & gallbladder
 - 3) Lining of Urinary bladder/most of urethra
 - 4) Respiratory system epithelium
 - 5) Thyroid, parathyroid, & thymus
 - 6) Tonsils
 - 7) Epithelium of tympanic cavity & auditory tubes

References

- Core References:
 - Ch. 5 & 6 in Ross and Pawlina, 2016. **Histology, A Text and Atlas**, 7th ed., A Wolters Kluwer publication. ISBN: 978-1-4511-8742-7
 - Ch. 4 & 5 in Mescher AL. eds. 2018. **Junqueira's Basic Histology: Text & Atlas**, 15th ed. New York, NY: McGraw-Hill publication.
 - Tao, LE, V. Bhushan and M. Sochat. 2018. **First Aid for the USMLE Step 1**; McGraw Hill Publication.

Lecture and Lab Feedback Form:

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